Sacituzumab govitecan (Trodelvy)

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

Antibody–drug conjugate · days 1 and 8 every 3 weeks · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Low blood counts / infection

Sacituzumab govitecan

Fever, shivering or suddenly feeling unwell needs immediate oncology advice; a low neutrophil count itself may cause no symptoms.

2 Diarrhoea / dehydration

Sacituzumab govitecan

Report loose stools early, especially with cramps, fever, dizziness or reduced urine. Follow the prescribed diarrhoea plan.

3 Nausea / vomiting

Sacituzumab govitecan

Use preventive medicines for both infusion days. Contact the team if you cannot keep fluids down.

4 Fatigue / anaemia

Sacituzumab govitecan

Tell the team about limiting tiredness, breathlessness or dizziness; blood tests help identify anaemia and other causes.

5 Hair loss

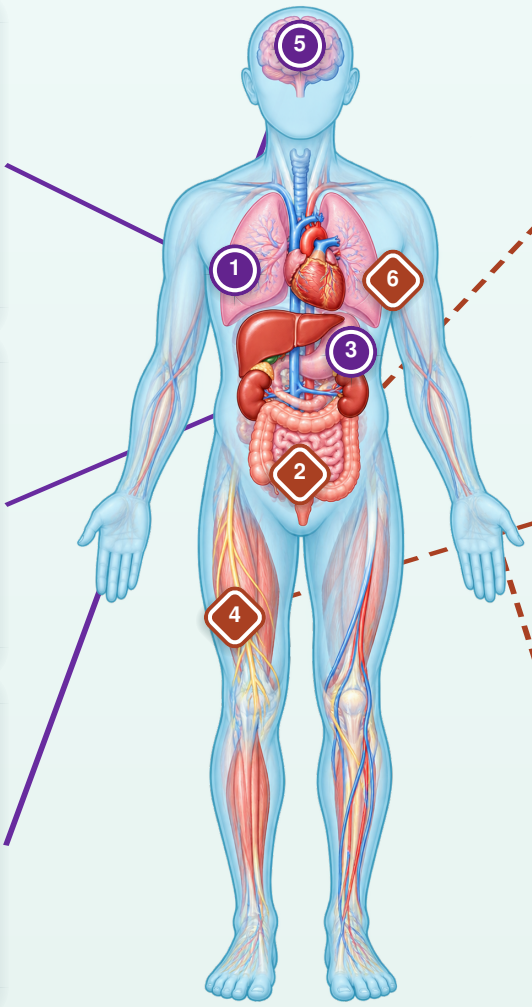
Sacituzumab govitecan

Hair thinning or loss can occur; ask about practical scalp and appearance support.

6 Infusion reaction

Sacituzumab govitecan

Tell staff immediately about rash, swelling, breathing difficulty or faintness during treatment.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology service immediately for fever, shivering or feeling acutely unwell; significant diarrhoea, inability to keep fluids down, dizziness, very little urine, or abdominal pain. Do not wait for the next treatment visit. During an infusion, tell staff immediately about breathing difficulty, swelling, rash, chest discomfort or faintness. Use the emergency service for severe breathing difficulty, collapse or other life-threatening symptoms. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Two treatment visits in each three-week cycle: Day 1, then Day 8. There is no planned infusion on Day 15. The next cycle starts after Day 21 if the team confirms it is appropriate.

The first infusion takes about three hours; later infusions usually take one to two hours if the earlier infusion was tolerated. Premedicines, checks and at least 30 minutes of observation after every infusion add to the visit. No take-home infusion pump is part of this regimen.

Checks and follow-up

Before the first treatment: full blood count, kidney and liver blood tests, glucose and electrolytes including magnesium, calcium and phosphate. Pregnancy testing is needed where relevant. electrocardiogram (ECG) is added if clinically indicated. The team checks infection, medicines, previous low blood counts and whether preventive granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) injections are appropriate. Blood count, kidney and liver tests are checked before each cycle; the neutrophil count must also meet the separate Day 8 threshold before that infusion. Glucose and the listed electrolytes are checked every second cycle. Pregnancy testing is repeated before each cycle where relevant. Review diarrhoea, hydration, nausea, oral intake, fever and infection before each infusion. Scans follow the individual response-assessment plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Prevent nausea before both doses

This is a high-emetic-risk regimen. Preventive antiemetics are needed for both Day 1 and Day 8, with take-home medicines and clear instructions. The steroid used as an infusion premedicine also provides antiemetic cover, so the team must reconcile the total dose.

A written diarrhoea plan

Report diarrhoea early. The team assesses infection, dehydration and electrolyte loss, and supplies an appropriate antidiarrhoeal prescription. Follow the written oncology instructions; do not improvise repeated high doses of loperamide.

Blood-count support

G-CSF injections may be considered from cycle 1 if the risk of febrile neutropenia is increased, or after significant neutropenia. Product and timing depend on the two-infusion schedule and the prescribing plan.

Infusion-reaction prevention

The unit gives premedicines to reduce reaction risk and observes you during and after each infusion. Tell staff immediately if you feel different or unwell.

Early cramping or diarrhoea during an infusion

This can be an acute cholinergic effect and needs staff assessment. Atropine may be used for later infusions when appropriate; tell the team if you have glaucoma.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00794/

Public information only. No personal information is encoded.